

Surgery for penile cancer

Surgery is one of the main treatments for penile cancer. The operation you have will depend on the stage of the cancer and where it is on the penis.

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About surgery for penile cancer

The operation you have depends on the stage of the [penile cancer](#) (cancer of the penis) and where it is on the penis. Your surgeon will try to remove all the cancer while keeping as much of the penis as possible.

There are different types of operation. Your surgeon or specialist nurse will talk to you about what surgery involves and the possible effects it may have. It is important to ask them any questions you have and talk about any concerns.

Some operations may cause changes to how your penis looks. They may also affect your [sex life](#). This can be difficult to deal with. Your team will explain any possible changes and give you support to help you cope.

Some types of surgery can affect your ability to make someone pregnant. This is called your fertility. If you are worried about your fertility, talk to your doctor or nurse before you start treatment. They may talk to you about storing sperm. We have more information about how cancer treatments can affect [your fertility](#).

Surgery to remove smaller cancers of the penis

There are 2 types of surgery to remove smaller cancers of the penis:

Glans resurfacing

You may need an operation called glans resurfacing if the cancer is:

- small
- low-grade
- only in the top layers of tissue on the glans (head of the penis).

In this operation, the surgeon will only remove the outer layers of tissue that are affected.

Glans resurfacing is usually done under general anaesthetic, during a short stay in hospital. Once they have removed the affected area, the surgeon takes a small [skin graft](#) from the upper thigh. They use this to cover the wound on the penis. This helps the wound to heal quicker. You will have a dressing to keep the wound clean.

During surgery, you will have a tube called a catheter inserted into your bladder. The catheter will drain your urine into a bag. This will make it easier for you to pass urine after surgery. You will also have a dressing on the thigh, covering the site where the skin graft was taken from.

The dressings and catheter can usually be removed after 7 to 10 days. This will allow time for the wounds to heal.

Circumcision

If the cancer is only on the foreskin, you may need an operation called a circumcision.

This is surgery to remove the foreskin. It can be done under local or general anaesthetic. You may be able to go home on the same day.

During surgery, the foreskin is removed just below the glans (head of the penis).

Dissolving stitches are used to attach the skin of the shaft to below the glans. These stitches usually dissolve within 2 to 3 weeks.

For the first 4 weeks after your surgery, you should avoid any sexual activity. This means intercourse and masturbation.

You will continue to get erections as normal after the surgery. But you may feel some tightness and discomfort around your scar. This usually improves within a few months.

Surgery to remove larger cancers of the penis

There are different types of surgery to remove larger cancers of the penis:

Wide local excision

If the cancer has spread over a bigger area, you usually need an operation called a wide local excision. This means removing the cancer and an area of healthy-looking tissue around it. This area of healthy-looking tissue is called a margin. This reduces the risk of the cancer coming back.

You usually have this operation under a general anaesthetic, during a short stay in hospital. The wound is usually closed with stitches. But some people have skin taken from another part of the body to cover the area. This is called a [a skin graft](#).

Glansectomy

If the cancer is bigger and on the head of the penis (the glans), you usually need to have this area completely removed. The operation is called a glansectomy.

A glansectomy is usually done under general anaesthetic, during a short stay in hospital.

Once the affected area has been removed, the surgeon takes a skin graft from the thigh. They use this to cover the wound on the penis. This can help make the penis look as similar as possible to how it looked before surgery.

Sometimes, instead of a skin graft, the surgeon might bring skin up from the shaft of the penis to cover the tip. Your surgeon can tell you which option is right for you.

During surgery, you will have a catheter inserted to make it easier to pass urine after surgery. You will also have a dressing on the thigh, covering the site where the skin graft has been taken from.

You need to stay in hospital for a few days after this operation.

The dressings and catheter can usually be removed after 7 to 10 days. This will allow time for the wounds to heal.

Your surgeon will do everything possible to make your penis look as similar as possible to how it looked before. But this type of surgery can change the size and shape of your penis. If you are [worried about this](#) and would like some support, let your doctor or specialist nurse know.

Removing part or all of the penis (penectomy)

If the cancer is large, or affecting too much of the penis, your surgeon may advise you to have:

- a partial penectomy, where part of the penis is removed
- a total penectomy, where all of the penis is removed.

They will talk to you about what is involved and how the operation will affect you.

Having a penectomy

If all of the penis is removed, the surgeon usually makes a small cut between the scrotum and anus (opening at the end of the bowel). This cut may be called an incision.

They may redirect the tube that carries urine to the incision. This tube is called the urethra. You are still in control of passing urine but you do this sitting down.

Sometimes it is not possible to redirect the urethra. Some people may need to have a tube that goes into the bladder through the tummy (abdomen). This is called a supra-pubic catheter. You attach a bag to the tube on the tummy to collect urine.

Having surgery to remove part or all of the penis can be upsetting. The team looking after you will give you lots of support. It is important to talk to your doctor or nurse about [your feelings](#) and concerns.

Reconstructive surgery after partial or total penectomy

If the cancer has not spread anywhere else in the body, it may be possible to have an operation to form a new penis. This is called reconstructive surgery. It may be done a year or more after treatment is finished. This is when you have recovered and there are no signs of the cancer coming back. It is very specialised surgery and is only done in a few hospitals in the UK.

The surgeon may take skin and other tissue from the arm or thigh to form a new penis. They can also put a small device called a penile prosthesis into the new penis. This will help you get an erection. Your surgeon or nurse will explain what the operation involves, so you know what to expect.

Surgery to remove the lymph nodes

If the penile cancer has spread to the [lymph nodes](#) in your groin, you may have an operation to remove them. This operation is called an inguinal lymph node dissection. It can be in one or both sides of the groin. The surgeon makes a cut in the groin and removes the lymph nodes.

This surgery is usually done under general anaesthetic, during a short stay in hospital.

During surgery, a small plastic tube called a wound drain is inserted near the operation site. This lets extra fluid drain away and helps the wound heal. It is usually removed after 2 weeks. But sometimes it may need to stay in place for up to 4 weeks.

The wound will have dissolving stitches, which take 4 to 6 weeks to dissolve.

After this operation, there is a risk of developing swelling in one or both legs, the scrotum and the penis. This is called lymphoedema. The risk is higher if you also have radiotherapy to the groin.

We have more information about [lymphoedema](#) and how it can be managed.

Before your operation

If you are having a general anaesthetic, you may need tests first. These are to make sure you are well enough for surgery. They can include [blood tests](#) and an [echocardiogram \(ECG\)](#) to check your heart. You usually have these at a pre-surgery assessment clinic.

If you smoke, you will be advised to stop or cut down before your operation. This will help you heal faster after surgery.

We have more information on [giving up smoking](#).

After your operation

How long it takes to recover depends on the surgery you had. If the cancer is very small, and only a small amount of skin is removed, your wound is likely to heal quickly.

After a glansectomy or partial penectomy



After a glansectomy or partial penectomy, the penis may be swollen and bruised immediately after surgery. This usually gets better after a few days. The nurses will give you regular [painkillers](#) until any [pain](#) or discomfort goes away.

Some people may find that their urine stream sprays when they pass urine. If you are having problems with passing urine after surgery, talk to your doctor or nurse. They can give you advice about what can help.

After surgery to remove the lymph nodes (inguinal lymph node dissection)

If the surgeon also removes the [lymph nodes](#), it may take longer to heal and recover. You will have a tube going into the groin to drain extra fluid. You may also have tubes going into the wounds to remove extra fluid and help them heal. The tubes are connected to a small bottle, which collects the fluid. A nurse removes the tubes after a few days, when fluid stops draining.

Sometimes fluid can build up in the groin area after lymph node surgery. This is called lymphoedema. We have more information about [lymphoedema](#) and how it can be managed.

Preventing blood clots

The nurses will give you elastic stockings called TED stockings to wear during the operation and for some time afterwards. This is to prevent [blood clots](#) in the legs.

After your operation, the nurses will encourage you to start moving about as soon as possible. This is important to help prevent problems, such as chest infections and blood clots. If your surgery includes a skin graft, they may advise you to rest in bed for the first 24 to 48 hours.

Urinary catheter

You may have a tube going into [the bladder](#). This is called a urinary catheter. It allows urine to drain from the bladder to outside the body. Your nurse usually removes it after a few days, but it may need to stay in longer. This will depend on which type of surgery you have.

Wound care

The nurses will check any wounds you have. They will change the dressings to keep the area clean. The stitches usually dissolve by themselves.

If you had a skin graft, the area the skin was taken from (donor site) may need dressings until new skin has formed. Your nurse will talk to you about this.

Some people may feel anxious about going out in public after surgery. It can help to do the following things:

- plan ahead and find a public toilet
- get a free [Macmillan toilet card](#)
- use disabled toilets.

Planning ahead can make you feel more confident. We have more information about [accessing public toilets and planning ahead](#).

Your surgeon will talk to you when the results of your operation are ready. This may be during your hospital stay or at an outpatient visit. They will tell you if you need any more treatment.

Your feelings after surgery

After surgery, your penis may be smaller and you may have scars. Changes in how you look and how your body works can be hard to cope with. This can directly affect your [sexual well-being](#). It can also affect how you feel about your body. This is known as your [your body image](#).

It is common to feel overwhelmed by [different feelings](#). It is important to tell your doctor and specialist nurse how you feel. They can get you the support you need.

Effects on your sex life

Surgery to remove a small cancer

Surgery to remove a small cancer is unlikely to have a direct effect on your sex life. Most people are usually able to have an erection and have sex.

Some people might feel less confident in their ability to have sex after penile cancer treatment. Even having minor surgery can be upsetting. Concerns about your sex life and intimate relationships are normal. If you are worried, speak to your doctor or specialist nurse. They can help you get the support you need.

Surgery to remove part or all of the penis (penectomy)

If you have surgery to remove part or all of the penis, you may worry about the effect on your sex life. Your cancer team will give you lots of support. They will explain what can help with sexual difficulties.

If you have had part of the penis removed, you may still get [erections](#). This will depend on how much tissue has been taken away during surgery. Your doctor or nurse can talk to you about this.

If the whole penis has been removed (total penectomy), your sex life will be directly affected. Not having a penis can be very difficult to come to terms with. But you may find new ways to enjoy sex. The areas around the scrotum and testicles will still be sensitive. You may still be able to have orgasms through stroking and other stimulation.

Organisations such as [Orchid](#) have further information about sex after penile surgery. They offer support for anyone affected by penile cancer.

The Macmillan Online Community also has a [penile cancer forum](#). You can talk to other people affected by penile cancer to support one another, ask questions, and share experiences.

About our information

References



Below is a sample of the sources used in our penile cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

EAU Guidelines: Penile Cancer. Available from: uroweb.org/guideline/penile-cancer (accessed May 2022).

Penile cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow up. Available from: [www.annalsofoncology.org/article/S0923-7534\(19\)31556-X/pdf](http://www.annalsofoncology.org/article/S0923-7534(19)31556-X/pdf) (accessed May 2022).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English

- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help